

Clover Health

February 23, 2026

TO: Dr. Thomas Keane, M.D., Assistant Secretary for Technology Policy, National Coordinator for Health Information Technology

FROM: Clover Health

RE: RIN 0955-AA13: HHS Health Sector AI RFI

Dear Assistant Secretary Keane:

Clover Health Investments, Corp. (Clover) appreciates the opportunity to respond to the Request for Information (RFI) regarding the adoption and use of artificial intelligence (AI) in clinical care. Clover is a physician-enablement technology company committed to expanding access to high-quality, affordable health care for people on Medicare.

Clover has been an industry leader in the development and responsible deployment of AI tools embedded directly into clinical workflows. Our experience demonstrates that AI can improve patient outcomes, reduce provider burden, and lower total cost of care—when deployed in a way that is clinician-led, longitudinal, and aligned with value-based care incentives. At the same time, public trust and regulatory certainty are essential to sustaining AI innovation in health care.

I. Executive Summary

Based on Clover's experience building and deploying AI tools in real-world clinical settings, we respond to the RFI in the following key areas:

- **Use of AI to Improve Health Outcomes:** Clover has been very successful using AI to improve health outcomes for the Medicare Advantage population. Clover's AI tools have been shown to (1) identify and treat disease earlier; (2) personalize care for patients; (3) improve prevention; (4) reduce acute care utilization, and (5) improve care and outcomes in underserved communities through earlier detection and reduced hospitalizations.¹

¹ Counterpart Assistant Whitepaper "Use by PCPs in Underserved Chronic Disease Populations Associated with Earlier Diagnosis and Less Frequent Hospitalization" December 2025. Available [here](#).

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- **Barriers to Private Sector Innovation:** Existing ambiguity in FDA guidance on AI in healthcare presents an easily fixable barrier. Specifically, to remove that barrier to healthcare AI innovation, the FDA should refine its Clinical Decision Support (CDS) guidance to ensure that “independent review” requirements do not mandate citations for medically self-evident diagnoses. By narrowing this construction, regulators can prevent administrative bloat and allow AI tools to prioritize high-value insights without being reclassified as regulated medical devices for performing basic data-to-diagnosis synthesis.
- **Enhancing Accountability and Data Rights:** To bridge the gap between AI accountability and data rights, HHS should clarify that “derived data”—the system-generated metadata and latent intelligence produced during model operation—remains the proprietary property of the developer. This distinction ensures that while patient inputs remain strictly protected, developers retain the essential diagnostic logs and performance metrics required to fulfill statutory safety monitoring and anti-bias obligations under emerging frameworks like the Colorado AI Act.

II. Background on Clover Health

Clover is a physician enablement technology company serving people on Medicare through Medicare Advantage plans and through our AI-powered clinical platform for physicians. Our model is built around patient choice, broad access, and clinician-facing tools that support better medicine.

- **Patient Choice and Affordability:** Our members choose their own providers through broad networks, and minimal in- v. out-of-network cost-sharing differences. We offer \$0 to low monthly plan premiums and copays with a maximum out-of-pocket limit.
- **Health Technology for All:** We empower clinicians to detect and manage chronic conditions earlier by equipping them with longitudinal patient data and AI-generated insights at the point of care and in their workflows. We do so through our technology platform, Clover Assistant, for in-network, primary-care physicians in our company’s MA plans, and through Counterpart Assistant for primary-care physicians in other plans. Clover Assistant and Counterpart Assistant provide the same platform (collectively here, “Clover Assistant”). Through Clover Assistant, we proudly provide the most sophisticated care-coordination tools (often available only to integrated, large health systems), to independent physicians’ groups in rural and underserved communities.

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- **The Right Care, at the Right Time, for Our Most Vulnerable Members:** Through our team of nurses, doctors, and other healthcare professionals, we identify high-risk members, and provide preventative care to them directly in their homes. And our teams use our cutting-edge AI platform, Clover Assistant, to provide superior care.

Below please find our responses to key areas outlined in the RFI. We welcome the opportunity to discuss how HHS can incorporate the use of AI tools in clinical care to further regulatory, reimbursement, and research and development approaches.

III. Use of AI to Improve Health Outcomes

Our AI platform is a non-medical device clinical decision support tool that analyzes a vast amount of data from over 100 different sources—including claims, labs, pharmacy, and electronic health records—to create a holistic view of each patient. Instead of a generic alert for a group of patients, AI provides personalized, evidence-based recommendations tailored to the specific person in front of the doctor. This approach enables our doctors to:

A. Identify and treat diseases earlier.

- **Diabetes:** Compared to those who did not have Clover Assistant visits with their primary-care providers, enrollees who had Clover Assistant-assisted provider visits: (1) primary-care practitioner use of Clover Assistant is correlated with earlier diagnosis (average HbA1c 6.65% for members having a Clover Assistant visit with a PCP and 6.91% for members not having such a visit); (2) were treated for diabetes an average of 36 months earlier; (3) were much more likely to obtain earlier treatment with oral diabetes medications and delay use of insulin at later stages of type 2 diabetes; and (4) had lower documented rates of hypoglycemia, a severe complication of diabetes, at 1,000 days (1.5% versus 2.4%).²
- **Chronic Kidney Disease:** Primary-care physicians who used Clover Assistant were able to diagnose—earlier—CKD stage 3 and higher. Earlier diagnosis of CKD resulting from CA-assisted visits slowed the decline of kidney function significantly (GFR, or glomerular filtration rate, of about 4.6 mL/min per year versus 1.3 mL/min per year), helping to delay and prevent adverse and costly health outcomes from reduced kidney function, including high blood pressure, heart disease, stroke, dialysis dependence, and death.³

² See [here](#) for more details.

³ See [here](#) for more details.

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B. Personalize care.

AI can surface best practices for complex chronic conditions, ensuring that even a physician who doesn't specialize in a particular area can provide relevant, up-to-date care. This is particularly important for patients with rarer conditions, where specialized expertise can be hard to find and where personalized treatment diverges from standard population-health protocols.

C. Improve prevention.

The platform prompts physicians to close critical care gaps and encourages bidirectional feedback through which the provider can instruct the plan to help close these care gaps. Together, these efforts contribute to an 11% higher rate of colorectal cancer screenings and 4.9% higher rate of breast cancer screenings.⁴

D. Less Frequent Acute Care Utilization.

- CA PCP attribution was associated with significantly less acute care utilization, including fewer all-cause inpatient hospitalizations (ranging from -7.6% to -21.2%) and 30-day readmissions (ranging from -11.5% to -20.8%) in members across all four studied chronic diseases.⁵
- Congestive Heart Failure (CHF) patients who had relationships with primary-care providers using the Clover Assistant had a lower average number of all-cause hospitalizations (18% lower) and 30-day readmissions (25% lower) compared to patients whose provider did not have access to Clover Assistant. Clover Assistant helped physicians facilitate more proactive and frequent longitudinal specialty care management for medically complex CHF patients who often face mobility challenges and reduced access to traditional office-based sites, including: (1) higher utilization of in-home care (22.8% versus 12.33% receiving at least 1 in-home visit); and (2) more outpatient cardiologist visits (1.1 visits for CA patients versus 0.8 visits for non-Clover Assistant patients).⁶

Clover's unique approach to the use of AI includes human-centered design, whereby clinicians can ignore AI-powered recommendations, there is no automated clinical action that the AI tool can take, and there is a built-in feedback loop that improves the model continuously.

⁴ Written testimony of Andrew Toy before House Energy and Commerce Committee, Sept. 3, 2025. Available [here](#).

⁵ See [here](#) for more details.

⁶ See [here](#) for more details.



IV. Barriers to Private Sector Innovation

The FDA's 2026 Clinical Decision Support (CDS) Software Guidance updates the 2022 guidance and clarifies the types of CDS functions that are excluded from the definition of a device. The FDA relies on four criteria for CDS to be excluded from the definition of a device:⁷

Criterion 1: The software is not intended to acquire, process, or analyze a medical image or a signal from an in vitro diagnostic device or a pattern or signal from a signal acquisition system.

Criterion 2: The software is intended for the purpose of displaying, analyzing, or printing medical information about a patient or other medical information.

Criterion 3: The software is intended for the purpose of supporting or providing recommendations to a provider about prevention, diagnosis, or treatment of a disease or condition.

Criterion 4: The software is intended for the purpose of enabling the provider to independently review the basis for the recommendations that such software presents so that it is not the intent that the provider rely primarily on any of such recommendations to make a clinical diagnosis or treatment decision regarding an individual patient.

The 2026 guidance expanded the 4th criterion for exclusion to state that the information should be presented in a way that prioritizes decision-relevant details and avoids information overload. The FDA also emphasized that recommendations must be grounded in "well-understood and accepted sources," such as clinical guidelines and peer-reviewed literature. This criterion should be narrowly construed in order to preserve the clinical accuracy of the provider. For example, if an individual has diabetes and their A1C indicates they have diabetes, Clover's AI tool should not have to link to clinical guidelines to confirm that the individual has diabetes. We urge the FDA to clarify that the use of clinical guidelines to confirm simple diagnoses should not be construed as an FDA regulated clinical decision support tool but an AI tool that is pulling an individual's data that the physician has the ability to confirm or reject the diagnosis.

⁷ Covington: *5 Key Takeaways from FDA's Revised Clinical Decision Support (CDS) Software Guidance*, Jan. 8, 2026, Available [here](#).



V. Enhancing Accountability and Data Rights

A critical legal issue surrounding the use of AI in healthcare is the assignment of ownership rights over secondary data. Derived data is a new creation resulting from the developer's proprietary logic, weights, and processing power. HHS should clarify that derived data created by an AI tool remains the property of the developer. This is distinct from the customer's underlying patient data (inputs), which remains subject to existing privacy and ownership protections.

Traditional healthcare contracts and Business Associate Agreements (BAAs) were designed for static software. They typically mandate the return or destruction of data upon contract termination. But continuous-learning AI requires access to secondary data—such as model confidence scores, latent representations, and processing logs—to improve accuracy, reliability, and safety over time. If developers are forced to “wipe” this intelligence per-deployment, the entire ecosystem suffers from stagnating model performance and a failure to learn from past errors.

There is a fundamental principle at stake: ownership of derived data must follow the responsibility for the model's performance. Under emerging frameworks like the Colorado AI Act, developers face heightened scrutiny and potential liability for “algorithmic discrimination” or harmful “hallucinations.” It is a functional impossibility for a developer to fulfill these legal obligations if they cannot retain the data necessary to:

- **Conduct Post-Market Surveillance:** Identify where a model may be drifting or failing in real-world settings.
- **Audit for Bias:** Aggregate processing data across diverse deployments to detect and mitigate disparate impacts.
- **Validate Mitigations:** Document that a known error has been successfully corrected through model tuning.

HHS should provide clear guidance that while underlying PHI and customer-specific inputs remain protected and owned by the covered entity, the derived analytical outputs and system-generated metadata belong to the developer for the purposes of monitoring, auditing, and safety-enhancement. Without this clarity, the “Duty to Monitor” imposed by regulators will remain in conflict with traditional healthcare contracts that do not give developers the ability to access the underlying data necessary to fulfill such duties.



VI. Conclusion

We support HHS's vision of adoption and use of AI in clinical care in a way that improves health outcomes, reduces barriers to innovation, and enhances accountability and clarifies data rights so that AI can be deployed most effectively.

To accelerate the adoption of AI in clinical care, HHS should prioritize these three critical areas based on Clover Health's experience:

- Use AI within clinical care settings to identify and treat disease earlier, personalize care for patients, improve prevention and reduce acute care utilization.
- Reduce barriers to private sector innovation, such as narrowly tailoring the applicability of the FDA's clinical decision support guidance.
- Focus on enhancing accountability and data rights by defining derived data to address liability and AI safety concerns.

Clover appreciates HHS' engagement on greater adoption of AI in clinical care and shares the agency's commitment to use AI effectively and in a way that fosters public trust. We look forward to continued collaboration and welcome further dialogue as HHS evaluates the use of AI in clinical care.

Best regards,

Signed by:

A handwritten signature in black ink that reads "William Chang".

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William Chang

Vice President and Head of Government Affairs